East of England Cancer Alliances

NHS

UROLOGY SUSPECTED CANCER REFERRAL FORM

Bladder, Kidney, Penile and Testis

Date of GP decision to refer: No. of pages sent:

NOTE: This form is NOT for use for patients under 16

INFORMATION PROVIDED TO PATIENT (To be provided by referring Clinician)	Please tick
Patient has been informed that cancer needs to be excluded	☐
Patient has been given written information leaflet regarding the Urgent Suspected Cancer pathway	☐
Patient understands that they may go straight to test	☐
Patient has confirmed they are available for the next 7 to 14 days	☐
Patient has confirmed they can accept earliest appointment and may be offered an appointment at any of the hospital sites	☐

PATIENT DETAILS – **Must** provide current telephone number			
Last name:		First name:	
Gender: Sex at birth *(Male, Female, Intersex*		DOB:	
NHS No:			
Address:			
Tele (Day/Work):		Mobile No:	
Email:		Patient happy for a message to be left	☐
Language:			

GP DETAILS	
GP name:	
Practice Code:	
Address:	
Telephone:	
Practice email: *(**ESSENTIAL** – to improve communication)*	

REASON FOR REFERRAL	
Bladder / Renal Cancer	
☐	**BLADDER / RENAL:** If >45 years AND unexplained visible haematuria without urinary tract infection
☐	**BLADDER / RENAL:** If >45 years AND visible haematuria persists or recurs after successful treatment of urinary tract infection
☐	**BLADDER:** If >60 years with unexplained non-visible haematuria ☐ Dysuria or ☐ Raised WBC count
Microscopic (NVH) haematuria: 2 of 3 urine dip or >12RBC on microscopy	
Penile Cancer	
☐	A penile mass or ulcerated lesion, where an STI has been excluded or already treated
☐	Unexplained or persistent symptoms affecting the foreskin or glans
Testicular Cancer	
☐	Non-painful enlargement or change in shape or texture

CLINICAL INFORMATION (Include any Radiology imaging if relevant)

Investigations		Results	*Dates:*
Creatinine	(within last 3 months to *support STT pathways)*		
eGFR	(within last 3 months to *support STT pathways)*		
UTI excluded	*Include Date treated*		
Mass detected on imaging? *(Attach report)*			
Ultrasound requested?			
Other, please give details:			
Patient does not meet NICE suspected cancer referral criteria, but you have a "gut feeling" that they warrant further investigation. ***Please provide full details in the clinical information section.***			☐

ROCKWOOD PERFORMANCE SCORE

Accurate functional status is needed to assess the most appropriate investigation/s and treatment.

If not known please select a score from the attached link for the Rockwood scale and put in the score box:
https://www.england.nhs.uk/south/wp-content/uploads/sites/6/2022/02/rockwood-frailty-scale_.pdf

SCORE:

The Rockwood Clinical Frailty Scale has not been widely validated in younger populations (below 65 years of age). If either apply please provide details of your patient's support requirements in the Additional Considerations section.

ADDITIONAL CONSIDERATIONS

Transport required?	☐	**If yes, please provide further details:**
Language / Hearing / Sight difficulties?	☐	
Learning difficulties?	☐	
SMI register?	☐	
Mental health issues that may impact on engagement?	☐	
Mental capacity assessment required?	☐	
Known safeguarding concerns?	☐	
Mobility requirements (unable to climb on/off bed)?	☐	

Mobility requirements	Walking	☐	Chair (Self Transfer)	☐	Chair	☐	Trolley	☐	AMB C1	☐	AMB C2	☐	AMB Car	☐	Own Chair	☐

BACKGROUND INFORMATION / RISK FACTORS						
BMI				Smoker/ex-smoker		
Alcohol				Other please specify		
Ethnicity						
Relevant family history						
Breast cancer	☐	Member		Age when diagnosed		Details:
Prostate cancer	☐	Member				

PATIENT MEDICAL HISTORY		
Existing conditions	☐	
Current medication	Acutes	
	Repeats	
Allergies	☐	
Anticoagulants / Antiplatelets	☐	
Diabetic	☐	
Immunosuppressant	☐	

Additional information / Resources:

Referral Criteria for Prostate (NG12) - Overview | Suspected cancer: recognition and referral | Guidance | NICE

Diagnostic uncertainty please use A&G / Advice & Refer pathway if locally available.

IF NHS E-REFERRAL IS UNAVAILABLE, COMPLETE FORM AND EMAIL TO THE REFERRAL TEAM WITHIN 24 HOURS.

To Note: These forms will be reviewed as part of regular cancer auditing